

FORMAL COMPLAINT OF UNPROFESSIONAL CONDUCT

Against: Lisa Mari — Responsible Pharmacist and Head of Strategic Team

Employer: Medscheme Holdings (Pty) Ltd — AfroCentric Health (JSE: ACT)

South African Pharmacy Council (SAPC) | Pharmacy Act 53 of 1974 | Good Pharmacy Practice Rules | CMS-REQUEST-3

To: The Registrar, South African Pharmacy Council (registrar@sapc.co.za)

CC: Professional Conduct, SAPC (conduct@sapc.co.za)

CC: Council for Medical Schemes — CMS-REQUEST-3 (info@medicalschemes.co.za)

CC: SANC Professional Conduct (professionalconduct@sanc.co.za) — for coordination with existing SANC complaint

From: Sr. Wilbur William Matthee (SANC No: 1583 1886) | info@ethichawks.co.za | wilburmat@gmail.com

Date: 16 February 2026 13 April 20256

Re: Unprofessional Conduct — Responsible Pharmacist — Use of Deficient Methodology to Dismiss Patient Safety Concern — False Final Resolution to CMS — Ongoing Patient Harm

1. INTRODUCTION AND COMPLAINANT STANDING

I am Sr. Wilbur William Matthee, a registered nurse (SANC No: 1583 1886), Specialist Nurse and Health Services Manager, holding qualifications of SRN, BTech Psych, Midwifery and Community Nursing, and a Postgraduate Diploma in Health Services Management at NQF Level 8.

During my employment with Medscheme Holdings (Pty) Ltd as Case Manager, I identified a critical system integration gap between the Gexus and DMS platforms (GEMS Ref: 000655789) that was generating false non-adherence reports and transmitting them to chronic patients registered with the Government Employees Medical Scheme. I formally proposed the necessary system correction through the appropriate internal channels. That proposal was rejected. The gap was not corrected.

I am filing this complaint in my capacity as a registered healthcare professional with a professional obligation to report conduct that poses a risk to patient safety and the integrity of the healthcare system. I am also a formally recognised whistleblower under the Protected Disclosures Act 26 of 2000, and this complaint is made in good faith on the basis of direct professional knowledge and documentary evidence.

2. THE RESPONDENT — LISA MARI

2.1 Professional Designation

The Respondent, Lisa Mari, holds the designation of Responsible Pharmacist and serves as Head of the Strategic Team at Medscheme Holdings (Pty) Ltd, a subsidiary of AfroCentric Health (JSE: ACT). AfroCentric Health administers medical scheme benefits for hundreds of thousands of beneficiaries across multiple schemes including the Government Employees Medical Scheme (GEMS).

The designation of Responsible Pharmacist is not a seniority title. It is a statutory designation established under the Pharmacy Act 53 of 1974, which imposes specific personal legal duties on the holder with respect to the accuracy, integrity, and clinical soundness of pharmaceutical and clinical information services under their oversight.

2.2 System Access and Professional Capacity

As Responsible Pharmacist and Head of the Strategic Team, Lisa Mari had direct professional access to both the Gexus and DMS platforms whose integration failure generated the false non-adherence reports. Her role required oversight of clinical data integrity across both systems. She had the professional capacity, the system access, and the statutory obligation to identify and investigate the structural cause of the data discrepancy rather than explain it away through claim pattern analysis.

3. THE GEXUS/DMS INTEGRATION GAP — THE PATIENT SAFETY CONCERN

3.1 Nature of the System Error

The Gexus system and the DMS (Disease Management System) platform used by Medscheme Holdings for chronic disease management of GEMS beneficiaries suffered from a structural integration gap. This gap caused the systems to generate false non-adherence flags — reports indicating that chronic patients had not adhered to their prescribed medication regimens — when in fact the non-adherence data was an artefact of the system's failure to correctly transfer and reconcile data between the two platforms.

The clinical significance of false non-adherence reports in a managed healthcare context cannot be overstated. Non-adherence reports for chronic patients can trigger scheme interventions, affect benefit authorisations, affect clinical case management decisions, and cause patients to doubt their own medication compliance — with potential downstream consequences for their health and their relationship with their treating clinicians.

3.2 The Complainant's Identification and Escalation of the Gap

I identified the Gexus/DMS integration gap in the course of my duties as Case Manager. I formally documented the gap, identified the necessary system correction, and submitted the correction through the appropriate internal clinical and management channels. This submission is detailed in Annexure C3 of the CMS Complaint Form (Pages 5-6), which forms part of the documentation in CMS-REQUEST-3.

The proposed system correction was rejected. No alternative corrective action was taken. The gap was not escalated to the relevant technical teams for resolution. Instead, the matter was closed internally without remedy.

4. LISA MARI'S CONDUCT — THE THREE-MONTH CLAIM PATTERN RESOLUTION

4.1 The Methodological Choice

The core of this complaint is not that Lisa Mari reached the wrong conclusion. It is that she chose a methodology that was structurally incapable of identifying the actual cause of the problem, and presented the result of that methodology as a final clinical resolution.

When the patient safety concern was escalated — whether internally or through the CMS complaint process — Lisa Mari formulated a response based on three-month claim pattern analysis. This methodology examined the pattern of claims submitted by the affected patient population over a three-month period and used apparent non-adherence patterns in that claims data to characterise the non-adherence flags as clinically explicable rather than system-generated.

4.2 Why This Methodology Is Professionally Deficient

The three-month claim pattern methodology is fundamentally incapable of distinguishing between genuine patient non-adherence and system-generated false non-adherence flags for the following reasons, each of which a Responsible Pharmacist with access to both systems would be expected to understand.

1. The methodology treats the system output — the non-adherence flags — as valid clinical data representing actual patient behaviour. But the very allegation being investigated was that the system output was structurally false. You cannot investigate whether a system is generating false data by analysing the data the system generated. That is circular reasoning.
2. A structural integration error between two platforms produces systematic errors that replicate consistently across the patient population. Three-month claim pattern analysis across a population affected by a systematic error will show consistent apparent non-adherence — not because patients are non-adherent, but because the error is consistent. The pattern that Lisa Mari's methodology found was the signature of the error itself, not the signature of patient behaviour.
3. The correct investigation methodology for a suspected system integration gap is a technical audit of the data transfer architecture between the two platforms — tracing specific patient records through the integration pathway to identify where the data discrepancy originates. This requires system access, not claims analysis. Lisa Mari had that system access. She chose not to use it.

A resolution that closes an investigation without correcting the error that prompted the investigation is not a resolution. It is a suppression. And a Responsible Pharmacist who formulates and presents such a resolution — with the system access and professional capacity to know that it is insufficient — bears personal professional accountability for that suppression.

5. STATUTORY AND REGULATORY DEPARTURES

The following table maps Lisa Mari's conduct against the specific statutory and regulatory provisions she is required to comply with as a Responsible Pharmacist registered with the SAPC:

Statutory / Regulatory Provision	Standard Required	Lisa Mari's Departure
Good Pharmacy Practice Rule 1.4	A pharmacist must ensure the accuracy and integrity of clinical information systems under their oversight.	Lisa Mari formulated a resolution that accepted structurally flawed system output as valid clinical data without investigating the architectural cause of the discrepancy.
Good Pharmacy Practice Rule 2.6	A Responsible Pharmacist must take personal responsibility for the clinical soundness of pharmaceutical services and data affecting patient care.	Lisa Mari accepted and presented as final a resolution that she had the professional capacity and system access to identify as clinically unsound.
Pharmacy Act s49 — Unprofessional Conduct	Conduct that is disgraceful or dishonourable or unworthy of a pharmacist, or that departs from the standards of professional practice.	Presenting a methodologically deficient analysis as a final resolution to a CMS patient safety investigation, while having access to the data that would have revealed the deficiency, constitutes a departure from the standards expected of a Responsible Pharmacist.
SAPC Ethical Rules Rule 8	A pharmacist must not allow any interest to influence their professional judgement to the detriment of patients.	The institutional interest in closing the CMS complaint without operational disruption to the claims management system was allowed to displace the professional obligation to investigate and correct a patient safety error.
CMS Act s26 — Administrator Duties	Administrators must ensure accurate and complete reporting of clinical information affecting beneficiaries.	The false non-adherence reports generated by the Gexus/DMS gap constituted inaccurate clinical reporting affecting GEMS beneficiaries. The resolution formulated by Lisa Mari perpetuated rather than corrected this inaccuracy.

6. EVIDENCE SUMMARY

The following evidence supports the allegations in this complaint and is available for production to the SAPC upon request:

Ref	Category	Description	Source / Annexure
EV-1	System Access	Lisa Mari held the position of Responsible Pharmacist and Head of Strategic Team with direct access to both the Gexus and DMS	Role designation — employer records

Ref	Category	Description	Source / Annexure
		platforms. Her role required oversight of clinical data integrity across both systems.	
EV-2	Knowledge of Gap	The Gexus/DMS integration gap (GEMS Ref: 000655789) was identified and formally documented by the Complainant and submitted through internal channels. As Head of the Strategic Team responsible for system integration oversight, Lisa Mari had professional access to the same systems and data sets that revealed the gap.	CMS Complaint Form — Annexure C3 (Pages 5-6)
EV-3	Methodological Choice	Rather than investigating the structural system architecture error, Lisa Mari formulated a resolution based on three-month claim patterns. This methodology treats the system output as valid patient behaviour data, which inherently precludes identification of a structural input error.	CMS Resolution — final response to Complainant
EV-4	False Final Resolution	Lisa Mari presented the three-month claim pattern analysis to the CMS as a final resolution of the patient safety concern. This closed the active CMS investigation without identifying, acknowledging, or correcting the underlying system integration gap.	CMS-REQUEST-3 — final resolution communication
EV-5	Ongoing Patient Impact	The Gexus/DMS integration gap continued to generate false non-adherence reports to chronic patients after Lisa Mari's resolution was accepted. The structural error was not corrected. Patients continued to receive clinically inaccurate information about their own medication adherence.	GEMS Ref: 000655789 — ongoing system state
EV-6	CMS Finding	The Council for Medical Schemes formally found Administrative Misconduct and Ethical Failure in relation to the handling of this matter and explicitly referred the matter to HPCSA and SANC for investigation. Lisa Mari's resolution was the instrument by which the misconduct was operationalised.	CMS Referral Letter — CMS-REQUEST-3

NOTE ON EVIDENCE: The Gexus/DMS system architecture records, the data transfer logs between the two platforms, and the internal communications regarding the proposed system correction are in the exclusive custody and control of Medscheme Holdings. These records are referred to in the Labour Court founding affidavit as documents required to be produced under rule 35 of the Uniform Rules of Court. The SAPC is requested to issue a formal request for these records to Medscheme Holdings as part of its investigation.

7. RELATIONSHIP TO PARALLEL PROCEEDINGS AND REGULATORY FILES

This complaint does not arise in isolation. It forms part of a documented pattern of conduct by Medscheme Holdings and its professional staff in which patient safety concerns were systematically suppressed and the professionals who raised them were retaliated against. The following parallel regulatory files are active and relevant:

- (a) SANC — Formal complaint against Registered Nurses Mary-Ann Jones and Mapule Mashele filed 10 February 2026 — four heads of professional misconduct including patient safety suppression
- (b) CMS — CMS-REQUEST-3 — finding of Administrative Misconduct and Ethical Failure — explicit referral to SANC and HPCSA
- (c) SAHRC — formal acknowledgement of seriousness of allegations regarding victimisation and financial prejudice
- (d) Labour Court Cape Town — founding affidavit ready to file — automatically unfair dismissal and constructive dismissal
- (e) CCMA — WECT23564-25 and WECT24486-25 — Certificates of Non-Resolution issued 9 January 2026
- (f) B-BBEE Commission — Case No: 05/02/2026 — Active
- (g) JSE Issuer Regulation — 325-page evidentiary bundle acknowledged — Active
- (h) Department of Employment and Labour — UIF complaint and follow-up escalation filed 16 February 2026

The SAPC is respectfully requested to coordinate its investigation with the CMS in respect of CMS-REQUEST-3, as the CMS finding of administrative misconduct and ethical failure directly concerns the same patient safety matter that is the subject of this complaint.

8. RELIEF SOUGHT

I request that the SAPC:

1. Open a formal investigation file and assign a case reference number and named investigator to this complaint within 5 business days of receipt.
2. Investigate whether Lisa Mari's conduct in formulating and presenting the three-month claim pattern resolution constitutes unprofessional conduct under section 49 of the Pharmacy Act 53 of 1974 and the SAPC Good Pharmacy Practice Rules.
3. Formally request Medscheme Holdings to produce the system architecture records, data transfer logs, and internal communications relating to the Gexus/DMS integration gap (GEMS Ref: 000655789) and the proposed system correction.
4. Coordinate with the Council for Medical Schemes in respect of CMS-REQUEST-3 to ensure that the CMS finding of administrative misconduct and ethical failure is properly incorporated into the SAPC investigation.
5. Investigate whether the ongoing generation of false non-adherence reports following Lisa Mari's resolution constitutes a continuing patient safety risk requiring urgent regulatory intervention.
6. Provide written acknowledgement of receipt of this complaint within 72 hours.

9. VERIFICATION AND DECLARATION

I declare that the contents of this complaint are true and correct to the best of my knowledge and belief. I make this complaint in good faith in my capacity as a registered healthcare professional with direct professional knowledge of the matters described, and in exercise of

my professional obligation under the SANC Code of Ethics to report conduct that poses a risk to patient safety and the integrity of the healthcare system.

I am a self-represented complainant currently experiencing destitution and homelessness as a direct consequence of the retaliation described in the parallel proceedings referenced above. These personal circumstances do not diminish the professional and patient safety significance of this complaint, nor my standing to make it.

Yours faithfully,

Sr. Wilbur William Matthee

Specialist Nurse | Health Services Manager

SRN | BTech Psych, Midw, Community | PG Dip HSM (NQF 8)

SANC Registration No: 1583 1886

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EthicHawks Consulting | 16 February 2026 **13 April 2026**

This complaint is tendered as Annexure SAPC-LM1 to the founding affidavit in the matter of Matthee v Medscheme Holdings, Labour Court Cape Town. It is simultaneously filed with the SAPC Registrar and copied to the CMS and SANC for regulatory coordination.